🧠 **APPLY** FAST assessment:
F — Face: ask child to smile (drooping? asymmetry?)
A — Arms: raise both arms (one drifts down?)
S — Speech: repeat a phrase (slurred? confused?)
T — Time: note exact onset time

Any FAST sign present, or other neurological signs?

N → 👁️ Continue monitoring. If symptoms develop or concern remains, call 101.

Y → 📱 **CALL** 101 immediately. State: suspected child stroke. Describe symptoms observed.

🕐 **NOTE** exact time symptoms began. If not witnessed: last time child was seen normal.

🧑 Is the child conscious?

Y → **LAY** child down with head and shoulders slightly elevated. **KEEP** calm and still. Loosen tight clothing.

N

🌬️ Is the unconscious child breathing?

N → ❤️ **BEGIN** CPR immediately. Follow MDA dispatcher instructions.

Y → **PLACE** in recovery position (on side). 👁️ Monitor continuously.

Is the child having a seizure?

Y → **CLEAR** area around child. After seizure: place on side. Note duration and type.

N

👁️ **MONITOR** continuously until EMS arrives. Watch for changes in consciousness and breathing. **PREPARE** for CPR.

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Reference Notes

DO NOT:

✕ **Do NOT delay calling 101** to wait and see if symptoms resolve.
✕ **Do NOT give the child any food or drink** — stroke may impair swallowing reflex, creating aspiration and choking risk.
✕ **Do NOT give any medications, especially not aspirin** — if the stroke is haemorrhagic, aspirin will worsen it.
✕ **Do NOT drive the child to hospital yourself** unless MDA explicitly advises it. MDA ambulances begin treatment en route.
✕ **Do NOT assume a child cannot have a stroke** — children can have strokes at any age.
✕ **Do NOT restrain the child during a seizure.**
✕ **Do NOT put anything in the child's mouth** during a seizure.
✕ **Do NOT rely solely on FAST to rule out stroke in children** — adult screening tools are not validated for the pediatric population.

Pediatric Stroke — Key Differences from Adults

- Seizures at onset are more common in children than adults
- Symptoms may appear gradually rather than suddenly
- Very young children may show only irritability, feeding difficulty, or lethargy
- FAST assessment is not validated for the pediatric population
- Treatment window: alteplase effective within 4.5 hours (ages 28 days–18 years)
- Mechanical clot removal may be effective for ages 6+ within 6–24 hours

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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Stroke	שבץ מוחי	SHEV-ets mo-KHI
Face	פנים	pa-NIM
Arm	זרוע	zro-A
Speech	דיבור	di-BUR
Seizure	פרכוס	pir-KUS
Headache	כאב ראש	ke-EV ROSH
Consciousness	הכרה	ha-ka-RA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child may be having a stroke	ייתכן שהילד סובל משבץ	yi-ta-KHEN she-ha-YE-led so-VEL mi-SHEV-ets
Can you smile?	אתה יכול לחייך?	a-TA ya-KHOL le-kha-YEKH?
Raise both arms	תרים את שתי הידיים	ta-RIM et shTEI ha-ya-DA-yim

Source: Magen David Adom (MDA) — MDA Stroke Recognition Protocol (FAST) and AHA 2024 First Aid Guidelines — Pediatric Stroke Considerations
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